

SPMM
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Revise

Forensic Psychiatry II

Paper B

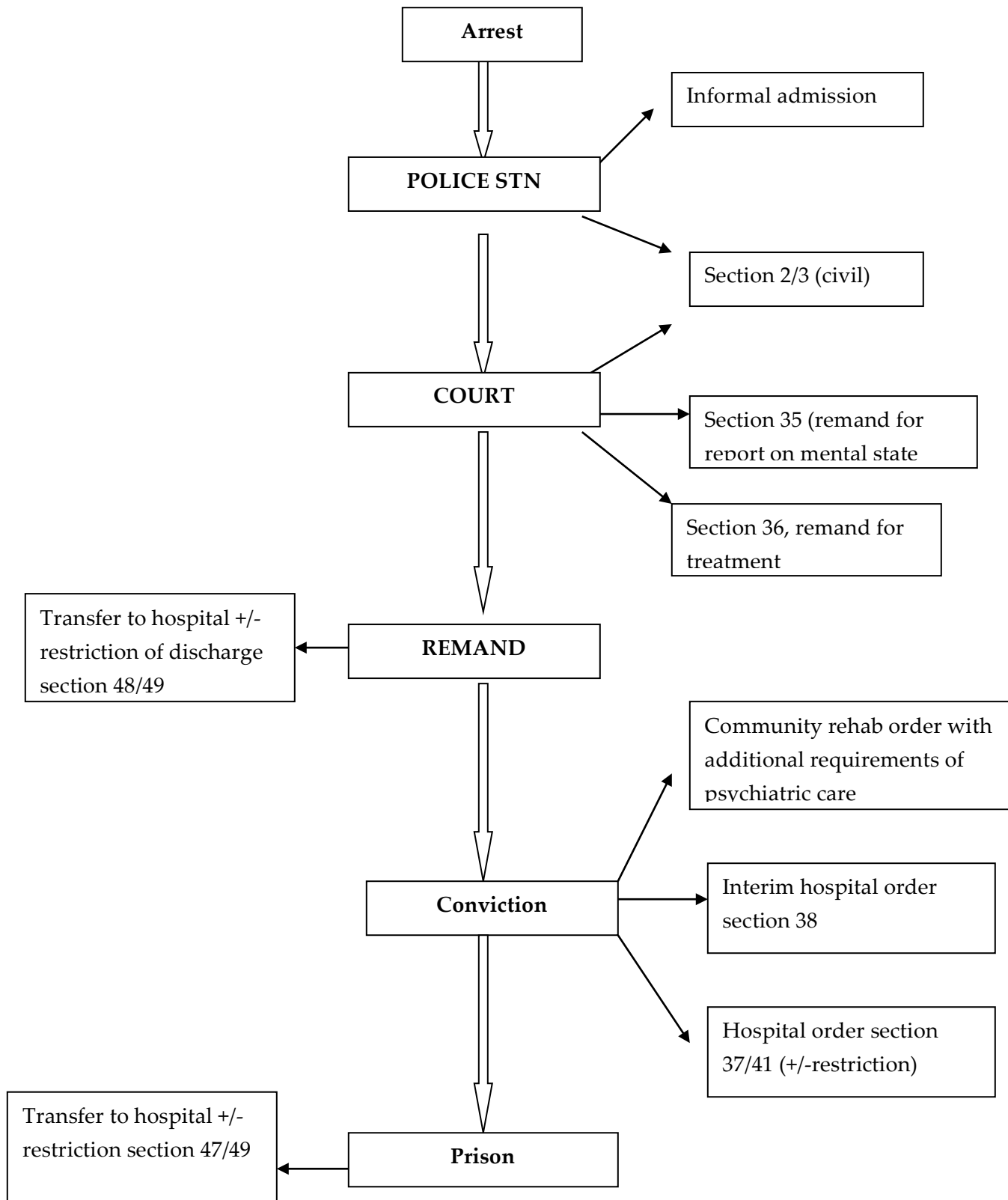
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1. Psychiatry and Criminal Justice System

MENTAL HEALTH ACT AND CRIMINAL JUSTICE SYSTEM IN ENGLAND AND WALES (PT)



A detailed knowledge of English/Welsh, Scottish or Irish legal system is NOT required for this section. The above flowchart has been provided only as a reference diagram for the concepts that are outlined below.

The pathway from an offence to eventual disposal is described below in accordance with the Justice system in England & Wales. Other jurisprudences have comparable pathways. Again, a detailed **knowledge of the exact sections and articles is NOT required** for the MRCPsych exam.

★ POLICE & ARREST:

- Police place the charges against a suspect.
- After a brief period of custody, the suspect is usually bailed with conditions until their court hearing. If the police do not grant bail, a court hearing takes place at the earliest possible time.
- If an individual is suspected to have a mental disorder or disability, an 'appropriate adult' should be informed and asked to attend the police station (Police and Criminal Evidence Act [PACE] guidelines).

★ COURTS & DIVERSIONS:

- The police send their information to the Crown Prosecution Service (CPS in England & Wales) who decides if the prosecution should continue. The CPS continue prosecution if the public interest outweighs any other concerns.
- After the first hearing, the individual can be bailed into the community (with certain conditions) or remanded into custody depending on the severity of the offence, amongst other factors.
- Magistrates or Crown Court can also choose to remand an individual to hospital for assessment of substance misuse and mental health [usually for a limited period, e.g. 28 days, up to a maximum of 12 weeks if Section 35 is used in England & Wales]. The purpose of this would be to provide the court with a report on the individual's mental disorder. [Crown Court can also remand to hospital for treatment under Section 36 lasting 28 days and renewable up to 12 weeks in total. This is rarely used (up to March 2007; there were only sixteen recorded cases of its use in England & Wales)].
- If Crown Court finds the accused unfit to plead, there will be a 'trial of facts' to decide whether the individual committed the accused act.

★ DISPOSALS:

- After a full trial or trial of facts, the accused may be either
 - (1) Absolutely discharged (acquittal),
 - (2) Placed on a probation or health supervision order (conditional discharge),
 - (3) Sentenced to prison or
 - (4) Disposed to hospital care (in some jurisdictions, a restriction order and/or a supervision or guardianship order may also be additionally imposed)

- A **supervision order** allows an individual to receive support and treatment, usually with the aid of a social worker. An absolute discharge may be considered if the offence is relatively trivial and there is no further action required.
 - After the court completes the trial hearing and sentences the individual, if they have a mental disorder a Section 37 '**guardianship order**' may be considered. Little use has been made of guardianship orders (probably due to the availability of supervision orders) but their purpose is primarily to ensure the offender receives care and protection rather than medical treatment in the community, although the guardian has the power to require the offender to live at a specific place, to attend specific places at specified times for medical treatment, occupation, education or training, and to require access to the offender to be given at the place where the offender is living to any doctor, approved social worker or other person specified by the guardian. The order would last initially for six months but the Responsible Clinician (RC) can also renew the section at the end of the first six months, again at the end of a second period of six months, and at yearly intervals thereafter.
 - In very serious cases where a hospital order has been given, the Crown Court may add a '**restriction order**' (section 41). This can be added to a hospital order and has no time limit. If a restriction is not added the order would last initially for six months but the Responsible Clinician (RC) can also renew the section at the end of the first six months, again at the end of a second period of six months, and at yearly intervals thereafter. If a restriction is added the RC would require the permission of the Ministry of Justice to allow leave or discharge from hospital, and the hospital managers have no power to discharge. Discharge could occur either absolutely or conditionally.
- ★ PRISONER CARE:
- If a sentenced prisoner (i.e the one who did not get an immediate psychiatric disposal, but sent to prison in the first place) requires transfer from prison to hospital, a transfer section (section 47) can be used. In practice, The Ministry of Justice, usually adds a 'restriction direction' (section 49), which prevents the hospital from discharging the individual.
 - In some jurisdictions, a sentenced prisoner can be detained in hospital for longer than the individual's sentence tariff. If the prisoner was given a fixed term sentence, the Section 49 restriction is removed after the expiry of the sentence: and the Section is converted 'notionally' to a Section 37.

2. Practicing psychiatry in a secure setting

A summary of the most important aspects of interaction between forensic psychiatry and judicial system is given here.

Forensic Institutions:

Apart from prisons, high-secure and medium secure forensic residential units where a high degree of psychiatric morbidity is often seen, places such as Young Offender Institutes (YOI) may also exist to cater specifically where young offenders (age 12-16) and juvenile offenders (age 15-17) may be homed. Detention schools, in general, have higher relational security than YOIs while secure children's homes have a lower level of security. In addition, adolescent secure hospitals provide medium secure placement options.

- High secure units are generally filled with MDOs who are of grave and immediate risk
- Medium secure provide services for high-risk MDO
- Low secure units generally hold mentally disordered offenders (MDO's) with limited violence/absconding risk

Other forensic bodies of importance:

Community Forensic Teams emphasis on step down in physical and procedural security while stepping up on therapeutic care. Various models have been utilized in the provision of care for mentally disordered offenders in the community.

- **Parallel Care Model:** Both the in-patient medium secure care and community care provided in parallel with the same service. Ensures full long-term community service that is continuous with inpatient care.
- **Integrated Care Model:** Upon discharge from low/medium secure units, general psychiatric services provide the longer-term rehabilitation. The general psychiatric teams will handle readmissions that do not require an escalation in security.
- **Hybrid Care Model:** Based on integrated services but includes a period of 'shared care' especially in the weeks/months after discharge. In such models, the forensic services may retain the long-term care for a few high-risk offenders.

Probation services: Probation refers to serving a sentence whilst in the community (not in a prison). Some offenders only receive a probation order as a disposal for their offences and are never sent to prison for that offence. But a large majority serve at least some prison time before coming into a probation period. Nearly 70% of supervised offenders in England & Wales are under probation when discharged from secure institutions.

The various roles of a probation officer include:

- Assessing offenders prior to sentence
- Monitoring progress of offenders that are sentenced in prison
- Advising the parole board
- Supervision of offenders released on licence & recall
- Supervising compliance with community sentencing & reporting breaches to court
- Advising offenders on services/resources locally
- Directly providing support e.g. via psychological programmes, etc

Inreach services: In recent times, **prison inreach service** has emerged as a major part of forensic service provision. Nearly 87% of prisons in the UK now have these teams, which also provide a link to the primary care health services.

National Offender Management Service – NOMS, joins up prison and probation services. NOMS conducts research/ advises the UK govt. on strategies and policies.

Multi-Agency Public Protection Arrangements (MAPPA): In England & Wales, mentally disordered offenders convicted of a specified sexual or violent offence and sentenced for longer than 12 months or detained in a hospital setting or on conditional discharge are deemed MAPPA-eligible. MAPPA provides a framework for interagency collaboration and communication to ensure successful management of violent and sexual offenders.

Therapeutic use of security:

There are three main areas to consider when determining the level of security relating to the psychiatric hospital:

1. **Physical** – fence/locks and other means of securing against leaving the premises. Also the use of soft cutlery, removing glasses and sharps and hanging points to reduce the risk of harm to self or others.
2. **Relational** – training, good communication, meaningful activities. Often a lack of relational security can decrease the actual safety net provided by the physical security measures.
3. **Procedural** – searches, investigating and learning from incidents, staff pre-employment checks, other clinical governance arrangements and risk management.

The level of security is generally determined by the balanced decision taken within a multidisciplinary forensic team setting, weighing up the need for safety vs. therapeutic objectives.

Security level (of the unit) should be continually assessed through planned observations, searches, etc.

Observation levels can be categorized as per the frequency of the observational act and the proximity of the observer.

1. General observation– location is known at all times, but not within eyesight.
2. Intermittent observation - at risk of violence/attempting suicide/
3. Within eyesight observation – high & imminent risk of violence /suicide,
4. Within arms length observation - high risk of acting very quickly – if opportunity arises – e.g. swallowing a small object within reach

Provision of temporary leave from secure units:

The decision for permitting temporary absence is generally based on:

- -Stability of mental state and risks
- -Insight
- -Rapport with staff
- -Engagement with treatment and rehabilitation
- -A patient's past behaviour whilst on leave

This is known as section 17 leave in England & Wales. Section 17 Leave forms should state purpose/conditions – e.g. the need for drug testing, and state the destination, availability of escorting the patient and means of transport. Please note that this leave is not the same as parole.

3. Prison psychiatry

- ★ Mental health problems are the most significant cause of morbidity in prisons. Over 90% of prisoners have a mental disorder.
- ★ Many prisoners have numeracy and literacy problems, and most prisoners have a lower than average IQ (Her Majesty's Inspectorate of Prisons, 1997b, 2000a; Singleton et al., 1998).
- ★ Mental disorder (including substance misuse diagnoses) was found in 37% of sentenced male prisoners, 63% of men on remand, 57% of sentenced women prisoners and 76% of women remand prisoners. Multiple diagnoses were commonly encountered, especially in remand prisoners: about one-quarter of men and one-third of women on remand received two or more diagnoses.
- ★ The frequency of psychiatric diagnoses in sentenced (Gunn et al., 1991) and remanded (Maden et al., 1995) prisoners is shown in the table below:

ICD 10 Diagnosis	Sentenced		Remand	
	Male	Female	Male	Female
Psychosis	2.4%	1%	6%	4.5%
Neuroses	6%	15%	28%	43%
PD	9%	16%	11%	15%
Subs Misuse	22%	30%	39%	41%

- ★ The most prevalent personality disorder diagnosis in each of the four prisoner sub-groups was DSM-IV antisocial personality disorder in men and the second most frequently encountered category was paranoid personality disorder.
- ★ Brugha et al. estimated the prevalence of probable functional psychosis in the past year was 4.5 per thousand (nearly 0.4%) in the household survey, while in the prison survey, the prevalence was over 10 times greater: 52 per thousand (0.52%). In prisons, one-fourth of all psychoses is attributable to psychoactive substances.
- ★ International data on morbidity: 62 surveys from prisons of 12 countries were reviewed by Fazel & Danesh, 2002. 3.7% of men had psychotic illnesses, 10% major depression, and 65% (61–68) a personality disorder, including 47% (46–48) with antisocial personality disorder. 4.0% of women had psychotic illnesses, 12% major depression, and 42% (38–45) a personality disorder, including 21% with antisocial personality disorder. Prisoners were several times (nearly 20 times) more likely to have psychosis and major depression, and about ten times more likely to have antisocial personality disorder, than the general population.
- ★ **Suicide and non-fatal self-harm**

- The suicide rate in prison more than doubled between 1982 and 1998, from 54 to 128 per 100 000 of the average annual prison population. Self-inflicted deaths currently represent about half of all deaths in prison. Local prisons bear the higher brunt of prison suicides.
- Prison suicides have risen sharply in United Kingdom in recent years.
- The most common method is hanging
- Suicide is nearly 8 times higher in prison population than general population
- The majority occurs within 6 months of imprisonment.
- Life-sentence prisoners are over-represented.
- Alcohol and substance misuse is the most likely psychiatric problem.
- Remand prisoners, young offenders, and those with a history of substance misuse and violent offences are at particular risk (Her Majesty's Inspectorate of Prisons, 1997b, 1999a).

4. Violence risk assessment

The Butler Committee equated dangerousness 'with a propensity to cause serious physical injury or lasting psychological harm'. It is a term no longer used and has been replaced with risk assessment and management.

The risk is the likelihood that harm will occur. **Risk assessment** is a process of scientific (statistical/clinical) calculation of likelihood of an adverse event; this includes specifying

1. What will happen?
2. When will this happen?
3. By whom will this happen?
4. How will this happen?

Risk management is integral to assessment; Assessment is not carried out to label people and categorise into groups – it is a dynamic process and specific to the event of importance. Risk cannot be eliminated but only reduced.

Inquiries such as the Ritchie report (England & Wales) on the killing of Jonathan Zito by the psychiatrically unwell Christopher Clunis identified failures of risk assessment and management by individual practitioners.

Problems with 'predicting' risk:

- ★ **Low base rate:** The events of interest are usually very rare. Hence, the predictive value is generally low. Serious violence is rare amongst the severely mentally ill disordered, killing or maiming of others is measured in probabilities of less than 1% (Wallace 1998). This low base rate seriously compromises the predictive utility of risk assessment because of the false positive rate of even an exceptionally good risk assessment instrument.
- ★ **Multifactorial:** Risk is dependent on several factors, which tend to change over time.
- ★ **Unknown interactions:** Comprehensive risk evaluations are time-consuming; often the degree and nature of interaction among various factors is unknown.

Risk factors for any untoward incident (suicide, crime or violence) can be categorized as static, stable and dynamic factors (Bouch & Marshall, 2003).

- ★ **Static** risk factors: These are fixed and historical: e.g. family history of suicide. They cannot be modified.
- ★ **Stable** risk factors: These are long term, enduring issues but are modifiable to some extent and not fixed: e.g. diagnosis of personality disorder.

★ **Dynamic** risk factors fluctuate markedly in both duration and intensity: the e.g. presence of acute anxiety symptoms or akathisia. A dynamic risk factor can act synergistically and multiply the effect of underlying static and stable risk factors if not addressed promptly. Certain dynamic factors may occur only in future (e.g. upon discharge, a patient may feel helpless).

A comprehensive risk assessment should consider static, stable, dynamic and future risk factors and include them in devising **risk management** strategies.

Homicide inquiries investigating the relationship between risk management and homicidal outcome have identified several problems that may be of critical importance:

- Failure to take carers view into consideration
- Undue emphasis on civil liberties of patients
- Failure to implement MHA properly
- Tendency to take cross-sectional rather than longitudinal view of risk
- Failure to share information

Approaches to risk assessment

Clinical approach

In this approach, a clinicians' subjective, intuitive judgment informed by experience and knowledge is used to estimate risk and guide decisions about treatment. But professional opinions are often highly variable for a given case and have poor predictive value, not supported by many policymakers. Most risk assessment currently performed by clinical methods. They are based on clinical experience, individuals knowledge and person-specific assessment to reach a conclusion. Only 1/3rd of pure clinical judgments are estimated to be correct in retrospective studies.

Actuarial approach

This approach is very popular in forensic services. This uses formal, algorithmic and objective procedures for quantifying risk as a numerical probability of a future outcome: e.g. patient A has a 60% chance of killing herself in the next 2 years, etc. It is found to be superior to other methods of predicting the risk of violence and sexual offending but does not inform the clinician much about the risk factors that require targeting to mitigate risk. There are hardly any actuarial tools available for self-harm and suicide risk, but there are many available for violence risk assessment.

Problems of actuarial tools include:

1. Historical aspects are given more importance – so a pessimistic view of risk with insensitivity to change results.
2. High false positive rates using these tools.

3. As mentioned above generalisation of actuarial tools developed in one setting to the other is difficult.
4. Further, actuarial approaches are often too focused on static and stable risk factors rather than dynamic and modifiable factors. Thus, though actuarial instruments more accurately identify at-risk groups, structured risk instruments may be more clinically useful for enhancing clinical decision-making.

Structured professional judgment

Structured professional judgment is an approach that aims to combine the evidence base for risk factors with an individual clinical assessment to complement psychiatric opinion. A structured, scales-based assessment is used when formulating risk management plan. Several risk assessment instruments are available to support structured professional judgment (e.g. HCR-20 to assess risk for violence in forensic settings)

It is important to note that neither actuarial nor structured judgments should replace conventional clinical assessment; they must be employed as an additional aide for systematically identifying and addressing relevant risk factors.

Stages in risk assessment

According to Bouch & Marshall, the following stages are identified for risk assessment and management.

- A. Identifying the need for a full structured risk assessment (not everyone will need this)
- B. Assessing static, stable, dynamic and future risk factors and considering protective factors
- C. Individual formulation of risk applied to the context of current presentation
- D. Considering possible interventions and the level of support required
- E. Anticipating the impact of possible interventions
- F. Developing a management plan with specified short and long term implementations
- G. Reviewing and revising the management plan with variations in risk factors.

Commonly used tools

Structured Risk Tools

HCR-20 (Webster): It is a popular structured clinical assessment tool for violence risk. HCR-20 (Historical, Clinical and Risk) shows good inter-rater reliability. It has

- ★ 10 historical items (history of previous violence, PCLR score, etc.)
- ★ 5 Clinical items (lack of insight, diagnosis of PD) and
- ★ 5 risk management items (feasibility of plans, lack of support, etc.)

It has been useful in predicting inpatient violence and community violence in discharged patients.

Historical items	Clinical items	Risk items
Previous violence history	Negative attitudes to health services	Management plan lacks feasibility
Young age at first incident	Active symptoms	Exposure to destabilisers (e.g. alcohol)
Unstable relationships	Impulsivity	Non-compliance
Major mental illness	Treatment unresponsiveness	Stress
Substance use	Lack of insight	Lack of personal support
Psychopathy		
Employment issues		
Personality disorder		
Early maladjustment		
Previous supervision failure		

SARA: Spousal assault risk assessment guide SARA is a 20 item set of risk factors for use in the assessment of spousal assault. It can be used to help gauge the risk of future violence in men arrested for spousal assault.

SVR-20 is a sexual violence risk 20 scale - this is a 20 item guide for assessing violence risk in sex offenders.

Actuarial instruments

Group data is obtained from high-risk individuals and then to applied to the patient in question. It gives a group risk, and it should be applied with caution. Different types include:

VRAG (violence risk appraisal guide – Quinsley 1995) entirely reliant on historical factors. Validated at Canadian prisons. It is made of 12 items and includes PCL_R as a subscale.

Violence risk appraisal guide	
PCL-R	Absence of schizophrenia (Presence is counted to decrease risk!)
Elementary school difficulties	Victim injury (minimal or none)
Personality disorder	Alcohol abuse
Younger age	Female victim
Separated from parents before age 16	Failed conditional release/supervision order
Never married	History of non-violent offence

Violence Risk Scale has 23 dynamic and 6 static variables.

PCL-R (Hare) is a scale to diagnose Psychopathy, informs risk assessment and treatment decisions. 0 – 40 score range; 0-2 for each item; 20 items in total. Cut off of 25 used to diagnose psychopathy. In the strictest sense, PCL was not designed to be an actuarial tool for risk assessment on its own.

The Static-99 is a ten item actuarial assessment instrument created by Hanson and Thornton, for use with adult male sexual offenders who are at least 18 years of age at the time of release to the community.

SORAG (Sexual risk offender appraisal guide) is a 14 item actuarial instrument that incorporates PCL_R.

5. Psychiatric legal defences

Actus reus: Refers to the act of crime

Mens rea refers to the intent of crime

The age of criminal responsibility is > 10 years in England & Wales; 8 in Scotland.

The different psychiatric defences are:

- ★ Before a trial – fitness to plead
- ★ During a trial – not guilty by reason of insanity, infanticide, automatism, diminished responsibility
- ★ During sentencing – psychiatric mitigation

Of these, the common use of the term defence refers most often to the trial defences.

1. **Fitness to plead:** Fitness to plead relates to the mental abilities to comply with trial proceedings. Physical conditions such as deafness / muteness are also relevant as if present, may result in unfitness to plead (but requires careful monitoring). There is a presumption of fitness and tests of cognitive ability, and the presence of neurological, psychotic or LD conditions are relevant.

Note: *R v Pritchard* criteria: This is often cited when discussing the 'test' for fitness to plead. An individual is unfit to plead if found incapable of:

- Understanding the charge/charges
- Deciding whether to plead guilty / not
- Exercising the right to challenge jurors
- Instructing solicitors and counsel
- Following the course of proceedings or
- Giving evidence in his/her own defence

These factors are relevant at the time of trial only (not for arrest or making charges).

Amnesia for the offence does not interfere with fitness to plead.

According to the Criminal Procedures Act 1964, 2 medical practitioners must give evidence in support of fitness to plead. If the fitness is raised as an issue prosecution, it must be proven 'beyond reasonable doubt', if raised by the defence, the evidence must be taken on the 'balance of probabilities'. It is possible to delay a trial on the grounds of the need for treatment to improve fitness to plead (FTP) status.

If a person is found to be unfit to plead, a trial can still occur, but, in this case, the proceeding is limited to a trial of facts. All of the various disposals are still possible following a trial of facts.

2. McNaughten Rules: 1843.

This has four components.

1. A defect of reason (impaired, not mere failure to exercise)
2. Due to a disease of mind (not externally caused)
3. Leading to loss of appreciation of nature and quality of an act (the offence; physical nature not moral).
4. So the accused did not realise what he was doing was wrong (either legally or morally).

The burden of proof lies with defence in such circumstances. The jury will decide if the defence is suitable. Flexible disposals are available in English courts for insanity defence, so the accused does not always get hospital confinement, if insanity defence is upheld.

3. Diminished responsibility defence reduces the charge of murder to a manslaughter. For this to be upheld, the defending counsel must demonstrate the absence of mens rea. Hence the act remains unlawful but no malice aforethought.

The disposal sentences for murder are restricted; but for manslaughter these are flexible. Females and those with no criminal records are more likely to get this defence granted than other groups of offenders.

A related notion is the capacity to form intent. Psychiatrists are often requested by court to review whether / not an accused could have formed a particular intent for the crime.

The legalities involve:

- i. Mens rea for the offence or recklessness
- ii. The specific jurisdictions e.g. the obscurity of a violated law may preclude a true intent

Symptoms preventing the formation of relevant mental thought processes:

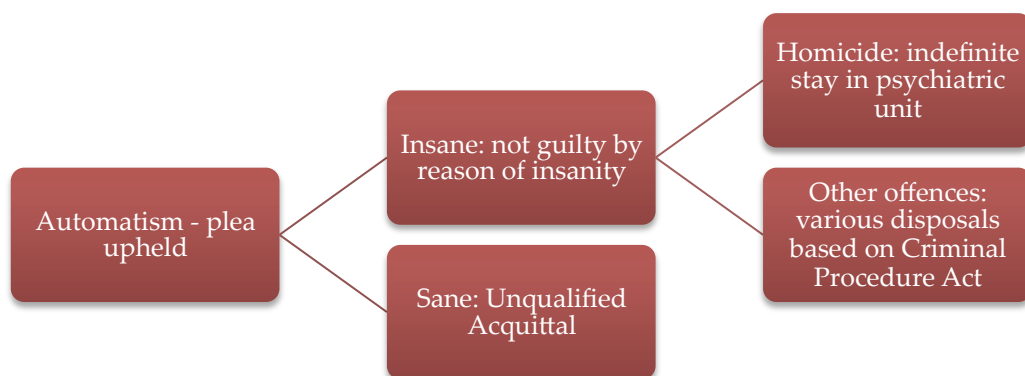
- Specific delusions relevant to the situation
- An abnormal mood state (severe depression or severe mania) or associated psychomotor changes, or impaired concentration
- Severe cognitive impairment – dementia/chronic schizophrenia

This does not include voluntary intoxication, which relates to doctrine of prior fault – it is expected that a person should foresee this.

If diminished capacity is upheld, an acquittal or a conviction for a lesser offence may ensue.

4. Automatism: It is a rare plea.

- ★ Automatism is a plea by a defendant that his actions were not under the control of his conscious mind, i.e. that his bodily movements were unwilled and involuntary. Although generally pleaded as a 'defence' a successful plea of automatism will negate the conduct element of the actus reus of any offence with which the defendant is charged.
- ★ Legal classification includes sane automatism where the act is one off, which if upheld, complete acquittal is possible, and insane automatisms that are likely to recur and thus incline towards psychiatric/medical disposals.
- ★ Automatism simpliciter (usually referred to as 'sane' automatism or simply 'automatism') is a complete 'defence' as the defendant is deemed not to be in any way to blame for what happened. Hypoglycemia (isolated), night terror and dissociative states can be classed as 'sane' automatisms. Here the cause is often 'external'.
- ★ Insane automatisms are likely to recur ('internal') and the counsel can plead not guilty by reason of insanity (insanity defence). Night walking, epilepsy, hypoglycemia due to a recurring condition are classed as 'insane' automatisms. There is no medical sense in these classifications
- ★ There are, broadly, three ways in which the courts have limited the defence of automatism. These are:
 - 1 Where the defendant exercised some control.
 - 2 Where the condition, which gave rise to the automatism, can be brought within the ambit of the rules on insanity.
 - 3 Where there was prior fault on the part of the defendant



Sleepwalking has often been regarded as sane automatism leading to acquittal in the past. In 1991, R vs. Burgess resulted in somnambulism being considered as a genetic and 'internal' problem that could repeat itself; hence it could be considered as sane or insane automatism depending on the Judge's discretion.

The following is a list of features that support sleep walking as an automatism defense: (Fenwick 1990)

- ★ Family / childhood history of sleep walking
- ★ Occurring within 2 hours of sleep onset (It is a non REM phenomenon)
- ★ Inappropriate behaviour with element of confusion witnessed
- ★ Presence of trigger factors such as alcohol, fatigue, sleep deprivation etc.
- ★ There is usually substantial amnesia
- ★ No attempts to conceal crime
- ★ Lack of sexual arousal, especially penile erection, when the alleged crime involves sexual nature. (from Lishman 3rd ed. Pg. 737)

5. Culpable homicide (an outcome of diminished responsibility argument): Lies in-between manslaughter and homicide. There is a lack of specific or evil intent to kill but murder has taken place. This can be **involuntary culpable homicide** where death is unintended death but occurs as a result of an assault or as a result of negligence. **Voluntary culpable homicide** refers to death resulting from intentional reckless act but because of provocation or diminished responsibility.

6. Intoxication is used as a defence very rarely. Involuntary intoxication refers to someone unwittingly taking a 'spiked' drink or automatism occurring as a side-effect to medical treatment. This constitutes a valid defence. But voluntary self-induced intoxication is not a defence. Apart from being 'spiked', involuntary intoxication may also include taking a drug prescribed by a medical practitioner, using a substance that is not dangerous but in a reckless manner, using a substance as a result of an irresistible impulse to take it (due to a dependent state). All of these can be used as mitigating factors to reduce sentencing or acquittal.

7. Mitigating factors:

These are factors that can reduce the culpability of the defendant. They include

- Being provoked
- Age or vulnerability
- Mental disorder or LD
- Involuntary intoxication
- Showing remorse
- Having a limited role in the offence

8. Amnesia

- This may indicate an abnormality at the time of offence

- Factors commonly associated with claims of amnesia (Kopelman 2002):
 1. Violence especially homicide (occurring 25-45% of occasions)
 2. Extreme emotional arousal
 3. Alcohol abuse and intoxication
 4. Depressed mood
- Psychosis, epilepsy, hysterical personality traits, parasomnias, organic brain disorder, hypoglycaemia and head injury may be relevant.
- The presence of amnesia has no legal implications in England and Wales in the absence of automatism.
- An issue of credibility may occur if the defence does not consider support of an expert witness. This may have a particular relevance to fitness to plead, fitness to be interviewed and reliability especially amnesic syndrome
- As highlighted earlier, amnesia around the offence does not render someone unfit to plead.

Other psychiatric aspects of legal proceedings

Fitness to give evidence: Concerns can occur that the witnesses are unfit – or unreliable, especially if at the extremes of age, suffer from psychiatric disorder or LD.

The commonly used test for fitness to give evidence is derived from case law. It includes

- Understanding the question
- Applying their mind to answering them
- Conveying the answers intelligibly to the jury

Confessions are generally seen as admissions of guilt. 3 types of ***false confessions*** are reported: voluntary false confession, coerced-compliant confession, coerced-internalised confession.

Factors influencing false confessions: (1) *Situational factors* – effects of custody/isolation – e.g. process of police interrogation (2) *Individual factors* – include mental disorder, incapacity etc.

An assessment for whether or not false confessions have been obtained involves reviewing transcripts- and assessing vulnerabilities, cognitive status, intelligence and mental disorder.

6. Role of a psychiatrist within the criminal justice pathway

The most important tasks include

- Assessing suspects brought under custody who have a high probability of being mentally ill
- Advising on mental health 'diversion' and attending to medical needs
- Advising on fitness to be interviewed and need for appropriate adult
- Advising regarding fitness to plead and stand trial
- Arranging transfer to hospital whilst on remand
- Providing solicited advice regarding sentencing options

Psychiatric reports to the Court

Good medical practice in writing reports

- 'You must be honest and trustworthy when writing reports.'
- 'You must do your best to make sure that any documents you write or sign are not false or misleading. This means that you must take reasonable steps to verify the information in the documents and that you must not deliberately leave out relevant information.'
- 'If you have agreed to prepare a report, complete or sign a document or provide evidence, you must do so without unreasonable delay.'
- 'If you are asked to give evidence or act as a witness in litigation or formal enquiries, you must be honest in all your spoken and written statements. You must make clear the limits of your knowledge or competence.'

(General Medical Council, 2006: paras 63, 65–67; Retrieved from

<http://apt.rcpsych.org/content/14/1/37>)

Professional and expert witnesses in mental health

There are 3 types of witnesses to court

1. An ordinary witness (witness to fact- not opinion)
2. A professional witness (to comment on clinical state, where they have had some involvement)
3. An expert witness (write reports or statements to court, regarding mental disorder or a specific issue)

Expert witnesses provide the following types of expertise as required by the court proceedings:

- Advisory
- Actuarial
- Clinical
- Experimental (evidence)

Human rights legislation

(For a detailed overview please refer to SPMM Smart Revise Notes on 'Psychiatric Services', section 5)

In the UK and EU, the ECHR (European Court of Human Rights) is enshrined in domestic law. There are several components or 'articles', although some are weighted differently (for example – see below). The Mental Health Act allows the state to detain people without the proof of committing an offence or having a trial. As a result of the complex nature of treating mental disorder compulsorily, there are occasional claims of abuse of mental health law by claimants and families. Historically, there have been concerns about politically motivated actions related to mental health law abuse and perceived over-consideration regarding dangerousness.

The articles that are most relevant to forensic practice are:

- Article 2 – the right to life –for example, the state has a positive duty to safeguard prisoners and others in custody.
- Article 3 – prohibition of torture and degrading treatment
- Article 5 – the right to personal liberty – Article 5 authorizes the detention of persons of 'unsound mind'. A requirement to comply with Article 5 brought about the creation of the 'deprivation of liberty safeguards'.
- Article 8- the right to private and family life – with a wide interpretation.

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